AnalystLab Africa Experience Lab | Week 6 | Data Analytics track

HealthConnect Rwanda

Healthcare operations and performance: what the data shows and what to do next

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Executive summary

9.31B
billed across 60,000 bills

90.1%
collected, with 920M still outstanding

1 in 3
visits is an emergency

8%
of bills can be tied to an appointment

A balanced network. The real differences sit in collections, staffing against demand, and gaps in the data itself.

What the data can and cannot support

5,000 of 60,000

appointments exist in the file, while billing, diagnoses and feedback reference up to APT060000. Facility, department and month views cover about 8% of bills.

- No admissions, lab, prescription or referral tables
- No facility names or locations
- Department matches facility in only 5% of appointments
- Billing status contradicts recorded payments

20,000 patients, evenly spread

50 / 50
men and women (10,090 and 9,910)

19.6 to 20.5%
of patients in each of the five provinces

45
average age, from 0 to 90

15 codes
insurance IDs of about 1,300 patients each; every patient has one

Coverage reads 100%, but with no reference table the IDs are best read as schemes, not proof of real cover.

Visits are flat across the year

400 to 440

appointments a month, no season

33%

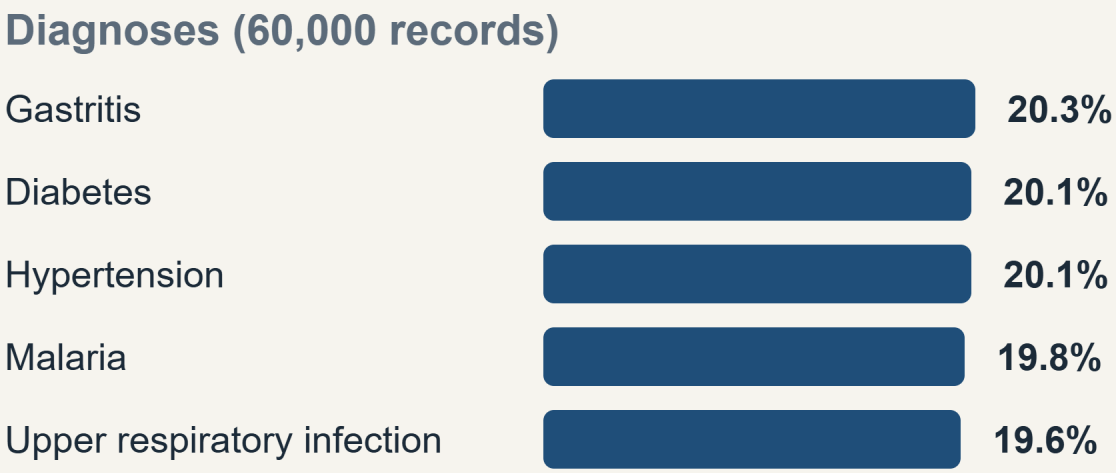
emergencies (1,650 of 5,000)

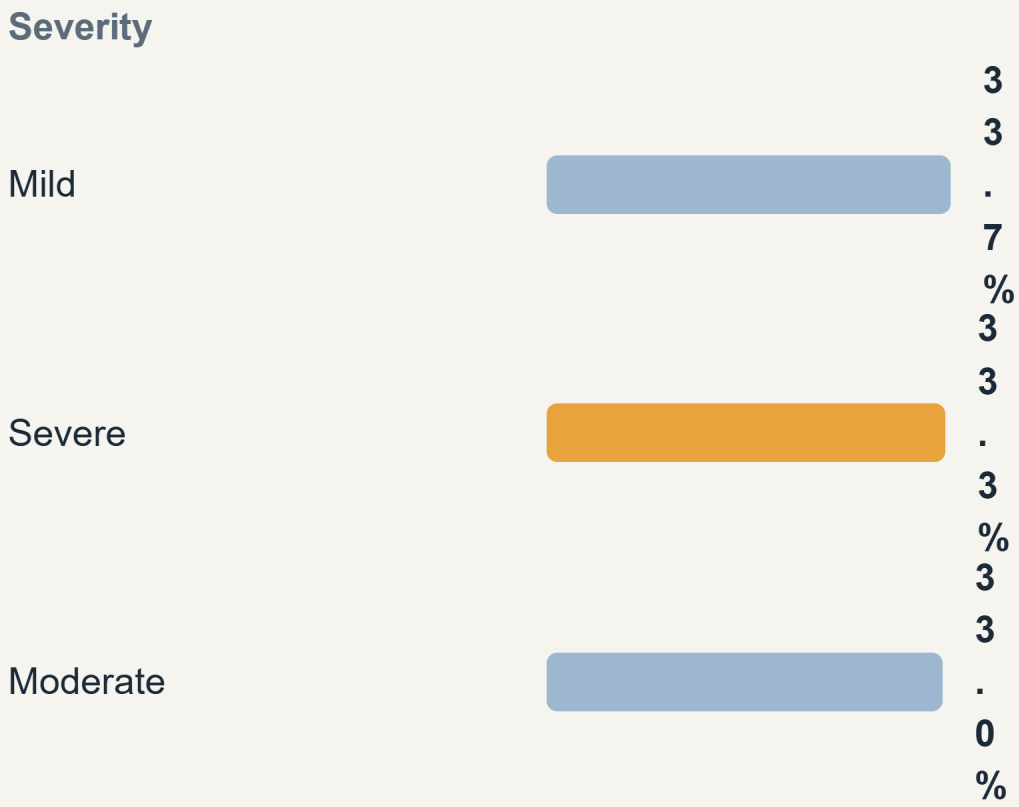
217 to 278

visits per facility, a 28% gap

Busiest facilities: HF013 (278), HF014 (276), HF010 (275), HF001 and HF011 (273). Quietest: HF019 (217).

Five diagnoses, each about a fifth of cases





Diabetes and hypertension together make up 40% of cases, which matters for long-term care planning.

Revenue follows visit volume

Facility	Billed (million)	Collection rate
HF011	44.2	90.1%
HF006	43.6	92.2%
HF013	43.5	90.7%
HF005	34.9	88.9%
HF018	34.8	89.8%

27%

gap between highest and lowest facility

11.2%

of billing from Cardiology, the top department

Linked bills only (5,000 of 60,000). Average bills sit between 145,000 and 163,000, so volume drives revenue, not price.

About 10% of the billed amount is outstanding

9.31B
billed

8.39B
collected (90.1%)

920M
outstanding on 11,887 bills

42,036 claims across four insurers. Approval sits between 49.3% and 50.9%; the rest are pending, none rejected.

Card 25.2%, mobile money 25.0%, insurance 24.9%, cash 24.8%.

Staffing is flat while demand is not

Facility	Visits	Staff	Visits per staff	Average rating
HF013	278	61	4.56	2.77
HF014	276	61	4.52	2.77
HF011	273	61	4.48	3.19
HF019	217	61	3.56	2.97
HF004	222	64	3.47	2.99

Every facility has 61 or 64 staff. Ratings rest on 72 to 102 reviews per facility, so gaps may be chance. HF013 and HF014 deserve a closer look.

Six recommendations

1

Tighten collections

Pursue the 11,887 bills with a balance and tie status to actual payments.

2

Match staffing to demand

Shift staff toward HF013, HF014, HF010, HF001 and HF011.

3

Investigate HF014 and HF013

Busiest and rated 2.77; HF014 also collects below average.

4

Review the emergency pathway

One visit in three is an emergency. Record why.

5

Set insurer targets

Half of all claims are pending. Start with Radiant.

6

Fix data capture

Load missing appointments, names, admissions, lab and pharmacy data.